



1. **What is the purpose of the MARCH trauma assessment?:** To find and treat any immediate life threats
2. **What patient fits the criteria for a rapid trauma assessment?:** A patient with an obvious life threatening MOI
3. **What are two common preventable but treatable trauma injuries?:** Obstructive shock from tension pneumothorax, and Exsanguination from an extremity hemorrhage
4. **When do we determine if a patient meets critical trauma criteria?:** As soon as the provider makes contact
5. **What is the fourth step in the MARCH Assessment?:** Assessment of the patient's circulation
6. **What does the MARCH assessment stand for?:** Massive Hemorrhage, Airway, Respirations, Circulation, Hypothermia
7. **You notice your patient is cool, pale, and diaphoretic, breathing rapidly and shallow at 37 RPM. You also notice blood pooling out of the patient's right anterior thigh. As a provider, what treatment comes first in the trauma setting?:** Finding and treating the hemorrhage.
8. **How and when should we request blood if needed?:** Early, and as soon as possible.
"Medic 99, requesting blood unit."
9. **How can we quickly determine a critical trauma patient?:** Mentation (AVPU), Skin condition, Respirations.
10. **In the back of a crashed minivan you notice two pediatric patients, one patient (M, 7y/o) Appears to have a broken finger, and is screaming loudly. His brother next to him (M, 5y/o) Appears to have a contusion on his forehead, and is staring in front of him silently, not reacting to your Presence. What patient demands a higher priority?:** The silent 5 year old. A healthy child should be noticeably reactive to injury, a silent child with a noticeable injury could be a sign of serious MOI and requires immediate intervention.
11. **About how long does it take to lose consciousness from an arterial hemorrhage?:** About 30 seconds, with death following just minutes after if left uncontrolled
12. **About how much blood can a fractured pelvis lose?:** Over 2 liters of blood
13. **A patient has an obvious hemorrhage from his right thigh spurting bright red blood, as a responder, how do you treat this per the MARCH assessment?:**
: Pack the wound and hold direct pressure for 3 minutes. Apply two tourniquets two as far up the leg as possible. Note the time, and check for pulses below the tourniquet.



14. **After packing a wound and applying a pressure bandage, your partner verbalizes that he is going to unwrap the bandage to check on the state of the wound. Is this advised?:** No. By unpacking the bandage you have released direct pressure, allowing for the wound to start bleeding again. Never undo a bandage or tourniquet that has been placed.
15. **How often should we evaluate a trauma patients airway?:** Every 10 minutes, or after every movement/patient intervention.
16. **After stopping a hemorrhage in a trauma placement and securing the airway, you notice the patient has no other life threatening injuries. During your interventions, you note the patient is showing signs of shock, with a weak pulse and pale diaphoretic skin. Is your patient ready for transport?:** No, the patient is showing signs of shock, and requires warming by the provider. Utilize a blanket to warm the patient before transport.
17. **What are three signs of cushings triad?:** Irregular RR, Bradycardia, and an increased BP
18. **Your trauma patient has a HR of 89 with a BP of 82/49, is this patient showing obvious signs of shock?:** Yes, a patient with a HR higher than their SBP is a clear indicator of shock.
19. **You and your partner are rushing to make contact with an unconscious patient who has been shot several times in a road rage incident. You equip your trauma bag, and primary care bag. Your partner is preparing the zoll monitor and the stretcher, should your partner continue?:** No, patients with an obvious and life threatening MOI do not need a 12 lead. Immediate patient contact and trauma care will save their life. Save the monitor for the secondary assessment.
20. **You are trying to send a trauma alert to dell seton. How should you go about this?:** By stating the patients age, gender, MOI, and criteria/interventions.
21. **In traumatic arrest, your partner immediately begins compressions after failing to find a pulse for 10 seconds. However, the patient has blood pooling from his brachial artery. Should your partner continue?:** No. Fix the hemorrhage first. Compressions cannot circulate blood that is leaking outside the body.
22. **What is the criteria for declaring a death/withholding?:** Obvious and extreme mortal wounds, Decapitation, Decomposition, Incineration, and a submersion over 20 minutes with a witness.
23. **Your pregnant patient has miscarried. Upon assessment, the patient tells you that the fetus was 25 weeks along. Is the fetus now considered a patient?:** Yes, any fetus over 21 weeks is considered a patient.



24. **How should we handle head trauma?:** Incline the head to a 45 degree angle, prevent hypoxia/hypertension. Avoid hyperventilating, and evaluate for cushings triad, dilated pupils, and seizures.
25. **How should we handle trauma to the chest wall?:** Evaluate structures of concern (Vessels, Lungs, and Heart.) Evaluate for flail chest, sucking v non sucking wounds, pneumothorax, and shock.
26. **How should we manage burn patients.:** Aggressively. Airway security is an immediate priority. Use dry dressings, and prevent hyperthermia. Apply EtCO2, and transport to dell.
27. **Who carries a cyanide kit?:** AFD Battalion Commanders.
28. **What is important to assess in traumatic falls?:** Did the patient hit their head, ore lose consciousness? Can they remember the event? Are they on Blood Thinners? Why did they fall? What was the height of the fall?
29. **How should we manage assaults?:** Determine what the patient was assaulted with, and how. (Closed Fist.) Determine where they were assaulted, and if they lost consciousness. Determine if the patient was strangled, or sexually assaulted. Did the patient lose control of their bladder?
30. **Treating a victim of assault can be very emotional for the victim, and the provider, and will likely lead to a court hearing. What can we do to ensure the victim has a chance for justice?:** Ask direct and necessary questions, document carefully and fully. Isolate the patient if needed, and get consent before every and any treatment.
31. **What three scenarios require ATCEMS medics to don a ballistic vest?:** Instruc-tions to stage, Riots/Civil Unrest, and any Forced Entry.
32. **What should we assess a patient for, before removing them from a crashed vehicle?:** Any spinal injury.
33. **What is the starting PEEP pressure for drowning emergencies?:** 5
34. **What three things make up the trauma triangle of death?:** Coagulopathy (increased lactic acid in blood), Acidosis (Decreased heart performance), and Hypothermia (decreased coagulation)
35. **How should we perform a proper blood sweep?:** With fingers closed like kitten paws
36. **In what order do we sweep for blood?:** Legs, Neck, Arms, Chest, Back, and Cracks
37. **When assessing a trauma patients respirations, what is a critical injury the provider should be assessing for?:** S/S of a tension pneumothorax
38. **When assessing head injuries in a trauma patient, what two conditions should the prover be assessing for?:** Hypoxia and Hypotension
39. **What is our target fluid resuscitation in general trauma patients?:** A MAP of 55-65.



40. **What is our target fluid resuscitation in general trauma patients with a suspected neurological injury?:** A SDP of 90 or above
41. **What does PAWS stand for?:** Pain management, Antibiotics, Wounds, and splinting.